PwC Digital Strategy

Common Theme Analysis

Comparison across all eight stakeholder interviews | Proposed healthcare acquisition | 31 October 2026

1. About this analysis

This analysis compares all eight interviews. The ordering of themes is for readability only. Mentions have not been counted and importance has not been ranked, because several of the narrowest points (for example the S4 conduct red flags or the S3 safety gates) could matter more than the broadest ones.

Everything below is qualitative, and the statements are unverified fictional-transcript statements. Where the transcripts do not answer a question, the report says "Not established in transcript".

ID	Stakeholder	Short name
S1	Chief Strategy Officer	Strategy
S2	Healthcare Commercial / Market Access Director	Commercial / Market Access
S3	Chief Medical Officer / Clinical & Product Quality Lead	CMO / Quality
S4	Regulatory & Healthcare Compliance Director	Regulatory
S5	CIO / Health Data & Cybersecurity Lead	CIO / Cyber
S6	CFO / Healthcare Deal Finance Lead	CFO
S7	COO / Supply Chain & Service Delivery Lead	COO
S8	Country / Regional Market Lead	Country Lead

2. Themes at a glance

#	Theme	Supporting stakeholders	Description
1	Large market is not the same as an accessible market	S1, S2, S8, S7	Market growth or forecast demand does not show that the target can win customers or deliver the volume.
2	Management claims need independent verification	S1, S2, S3, S4, S5, S6, S8	Seven stakeholders ask for original, independent or customer-sourced evidence in place of claims, pitch material or policy documents.
3	Transfer of relationships, reputation and advantages	S1, S2, S8	The acquisition thesis may assume the target's advantages travel to new markets and survive a change of ownership.
4	Regulatory and approval pathway affects timing and cost	S4, S8, S1, S2	Licences, registrations and local steps are country-specific and may delay entry.
5	Patient safety, quality and continuity during integration	S3, S7, S5	Ownership, supplier, system or process changes could disrupt safe service, product availability or records.
6	Health data, cyber and system integrity	S5, S2, S6	The target's data practices and system resilience need examination, and technology could block adoption.
7	All-in cost, remediation and downside	S6, S3, S4, S5, S1	The headline price may understate remediation, upgrades, working capital and integration costs. Several stakeholders ask for remediation to be owned and costed.
8	Most answers depend on what the target actually is	S2, S3, S4, S5, S6, S7	Several stakeholders say that evidence needs depend on the target's business model.
9	Conditions for pausing or escalating	S1, S3, S4, S6	Several stakeholders set their own triggers. They are different but overlapping.

3. Theme detail

1. Large market is not the same as an accessible market

Description	Market growth or forecast demand does not show that the target can win customers or deliver the volume.
Supporting stakeholders	S1, S2, S8, S7 (from the capacity side)
Supporting quotes	S1: "A broad statement that healthcare demand is growing is not enough to justify this target or its valuation." S2: "Market growth does not automatically become revenue for one company." S8: "Do not equate a large healthcare market with an accessible market." S7: "A market forecast does not tell us whether the target can safely deliver the extra demand."
Contradictory evidence	None direct. S1 only says the "attraction is speed" and does not argue market size is sufficient.
Potential impact	A top-down market-size case could overstate revenue and the valuation that rests on it.
Evidence gaps	Target segments, countries, purchasing process, competitor map, and the target's capacity. Not established in transcript.

2. Management claims need independent verification

Description	Seven stakeholders ask for original, independent or customer-sourced evidence in place of claims, pitch material or policy documents.
Supporting stakeholders	S1, S2, S3, S4, S5, S6, S8
Supporting quotes	S1: "We should show what we know, what management claims, and what remains untested." S2: "rather than relying only on the target's pitch deck." S3: "Marketing material and testimonials alone do not establish clinical benefit." S4: "We should reconcile management's answers with original records." S5: "A policy document alone does not prove controls work." S6: "Reconcile reported revenue to contracts, invoices, cash collections and underlying activity." S8: "customer evidence rather than management optimism."
Contradictory evidence	None. This is the most consistent view across the interviews. S7 does not use this framing, but asks for data and trends ("Definitions and historical trends matter more than a one-off snapshot.").
Potential impact	Diligence that relies on management representations could miss issues in quality, compliance, cyber or earnings.
Evidence gaps	What management has actually claimed is Not established in transcript. No claims or source documents were supplied.

3. Transfer of relationships, reputation and advantages

Description	The acquisition thesis may assume the target's advantages travel to new markets and survive a change of ownership.
Supporting stakeholders	S1, S2, S8
Supporting quotes	S1: "the target's reputation and capabilities do not transfer to our intended markets". S2: "The team may assume that existing relationships automatically travel across

	borders". S8: "whether the relationships are institutional or personal".
Contradictory evidence	None direct. S1 frames speed and access as the attraction, which depends on this transfer. S1 also says the advantages are not yet shown, so the views are compatible.
Potential impact	If advantages are not exclusive or do not transfer, the case for buying over partnership or organic entry could weaken. Customer loss after closing could affect revenue.
Evidence gaps	Change-of-control terms, customer retention, exclusivity of advantages, key-person dependence. Not established in transcript.

4. Regulatory and approval pathway affects timing and cost

Description	Licences, registrations and local steps are country-specific and may delay entry.
Supporting stakeholders	S4, S8, S1, S2
Supporting quotes	S4: "We must not assume one country's approval works everywhere." S8: "Local registration or licensing steps, procurement cycles, reimbursement decisions where applicable, onboarding requirements, customer approvals, language and service expectations, or gaps in local support." S1: "required approvals or access channels are uncertain". S2 lists "tender rules, reimbursement or budget constraints".
Contradictory evidence	Tension (consultant inference): S1 gives "speed" as the attraction, while S4 and S8 describe delays that could slow benefits. No timeline is given.
Potential impact	Launch dates and cost in the forecast may be unrealistic if the approval pathway is not modelled.
Evidence gaps	Target's products, countries, licences, regulator history and the time and cost of missing approvals. Not established in transcript.

5. Patient safety, quality and continuity during integration

Description	Ownership, supplier, system or process changes could disrupt safe service, product availability or records.
Supporting stakeholders	S3, S7, S5
Supporting quotes	S3: "A change in ownership, supplier, system, service location, or clinical process can affect availability or continuity." S7: "Changing suppliers, systems, processes or leadership too quickly can disrupt availability, care workflows, service levels or quality." S5: "Poor migration or interoperability can interrupt workflows, make records unavailable, break interfaces, or compromise data integrity."
Contradictory evidence	Tension (consultant inference): S7 warns against changing things "too quickly", while S1 emphasises speed. S1's "speed" refers to entry, not integration, so this is not a direct conflict.
Potential impact	Integration choices could affect patients, customers and regulatory standing, and so also revenue and reputation. Stop authority and stable-state definitions would matter.
Evidence gaps	Critical products and services, dependencies, integration plan, and who can stop a change. Not established in transcript.

6. Health data, cyber and system integrity

Description	The target's data practices and system resilience need examination, and technology could block adoption.

Supporting stakeholders	S5, S2 (data-protection teams as a blocker), S6 (cyber cost in the cost list)
Supporting quotes	S5: "A policy document alone does not prove controls work." S2: "clinicians and data-protection teams can still block implementation". S6: "technology and cyber upgrades".
Contradictory evidence	None. Only S5 discusses this in depth.
Potential impact	Cyber or data-rights issues could add cost, delay implementation or restrict data use, including any AI opportunity.
Evidence gaps	Whether the target holds health data, system inventory, breach history, test results. Not established in transcript.

7. All-in cost, remediation and downside

Description	The headline price may understate remediation, upgrades, working capital and integration costs. Several stakeholders ask for remediation to be owned and costed.
Supporting stakeholders	S6, S3, S4, S5, S1
Supporting quotes	S6: "liabilities that are not obvious from the headline valuation." S3: "uncosted remediation plan". S4: "a remediation requirement with no credible owner, cost, or timeline". S5: "Cyber remediation, licence and vendor costs, data migration, interoperability, system validation where required, resilience improvements, and the time needed to integrate or replace platforms." S1: "a downside case".
Contradictory evidence	None. Interviewees raise remediation in different domains, and no remediation need is identified.
Potential impact	Returns could be lower than a headline-based model suggests. The downside case would need to cover quality, regulatory and cyber costs.
Evidence gaps	No cost or valuation figures are in the transcripts. Not established in transcript.

8. Most answers depend on what the target actually is

Description	Several stakeholders say that evidence needs depend on the target's business model.
Supporting stakeholders	S2, S3, S4, S5, S6, S7
Supporting quotes	S2: "That depends on the business model." S6: "That depends on the target." S3: "The precise list depends on what the target actually does." S4: "First, list the target's actual products and services and every country in which it operates or plans to enter."
Contradictory evidence	None.
Potential impact	Without the business model, no risk or opportunity can be sized or prioritised.
Evidence gaps	Target type (care provider, product company, health-tech), countries, products. Not established in transcript.

9. Conditions for pausing or escalating

Description	Several stakeholders set their own triggers. They are different but overlapping.

Supporting stakeholders	S1, S3, S4, S6
Supporting quotes	S1: "I would also pause if we cannot explain why acquisition is better than partnership or organic entry." S3: "should be escalated before commitment". S4: "These issues need legal and regulatory review before the team recommends proceeding." S6: "the recommendation may need to be conditional or the decision paused."
Contradictory evidence	Difference: S6 allows a "conditional" outcome, while S1, S3 and S4 describe pausing or escalating. This may reflect different roles, not disagreement.
Potential impact	Without a single agreed gate framework, stakeholders may apply different thresholds to the same facts.
Evidence gaps	Whether any trigger currently applies, and the client's investment thresholds. Not established in transcript.

4. Differences and contradictions

- No interviewee directly contradicts another. This is a finding from comparing the eight transcripts, and it does not mean the views are validated.
- The two tensions (speed against approval delays, and speed against integration pace) are inferences. The transcripts give no timelines to test them.
- Only S1 raises partnership or organic entry as an alternative to acquisition. The others do not address it, so it is neither supported nor opposed elsewhere.

5. Unique observations

Stakeholder	Unique observation
S1	Avoid counting the same benefit under both revenue growth and synergy.
S2	The sales force may not be able to cross-sell immediately, and customers may leave on a change of control.
S3	Independent review and clear accountability for remediation.
S4	Conduct risks such as interactions with healthcare professionals, promotional claims, tender conduct and anti-bribery controls.
S5	Any AI opportunity is a hypothesis until data rights, workflow fit, safety, oversight and benefits are established.
S6	Client-supplied investment thresholds are needed.
S7	A Day 1 package and a 100-day plan with explicit safeguards.
S8	Dependence on a founder or a few individuals, and language and service expectations.

6. Implications for the business

- The evidence supports a diligence plan, not a decision. Every theme is a question to answer, and none is a reported finding.
- Themes 2 and 8 shape how diligence is run. Establish the target's business model first, then require independent evidence.
- Themes 3, 4, 5 and 7 are where the case could change. They touch exclusivity, timing, safe operations and cost, and should be tested before large spend.
- No acquisition recommendation can be made from these interviews alone.

7. What needs further validation

- The target's identity, business model, countries and products.
- Contracts, including change-of-control terms, and customer references.
- Licence, inspection and quality records.
- System, data and cyber evidence.
- Reconciled financials and the client's thresholds.
- Interviews with target management, customers, local partners and legal and quality reviewers.